Insulin Types & Timing

Knowing onset, peak, and duration helps you plan meals, corrections, and overnight coverage. Brand names vary — match this chart to what your clinician prescribed.

Action profile overview

TYPE	EXAMPLES*	ONSET	PEAK	DURATION
RAPID	Lispro, aspart, glulisine	**10–20 min**	**1–3 hr**	**3–5 hr**
SHORT / R	Regular (U-100)	**30–60 min**	**2–4 hr**	**5–8 hr**
INTERMEDIATE	NPH	**1–3 hr**	**4–8 hr**	**12–16 hr**
LONG	Glargine, detemir	**1–2 hr**	**Minimal**	**~20–24 hr**
ULTRA-LONG	Degludec, U-300 glargine	**~1 hr**	**Minimal**	**36–42+ hr**

*Examples of classes — not a prescribing list. Timing varies by dose, site, and person.

Common roles

- **Bolus (meal/correction):** rapid or short-acting
- **Basal (background):** long or ultra-long; sometimes NPH
- **Premixed:** fixed basal + bolus ratios
- **Pump / AID:** rapid-acting for basal & bolus

Safety reminders

- Never share pens or needles
- Store unopened insulin as labeled
- Track in-use pen expiry dates
- Rotate injection sites
- Clarify cloudy vs clear products before injecting

Stacking risk

Giving another correction before the previous rapid insulin has peaked can cause delayed hypoglycemia. Follow your clinician's correction spacing (often ~3–4 hours) and "insulin on board" guidance.

Educational reference only — not medical advice. Insulin regimens are individualized. Do not change doses without clinician direction.